Capecitabine

Tablet chemotherapy for breast or colorectal cancer; confirm your treatment aim.

Colorectal and breast cancer · NCCP 00216 v8 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

My overall treatment

Capecitabine twice daily on Days 1–14, followed by seven days off. Adjuvant treatment is usually eight cycles; advanced treatment continues according to benefit and tolerance.

Days 1–14
Take tablets with food or within 30 minutes afterwards.

Days 15–21
Planned tablet break; symptoms may continue.

A typical 21-day cycle

Day 1–14	Day 15–21
Treatment days	Planned tablet-free days

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Blood count, kidneys, liver and DPD testing** Required by regimen	DPD before first treatment unless already tested
During	**Blood count, kidneys and liver** Required by regimen	Before each cycle; earlier if needed
Review / after	**Response, recovery and follow-up** Team-agreed plan	Timing depends on treatment setting

Between treatments

- Stop tablets and call for significant bowel, mouth or hand-foot symptoms.
- Chest pain needs urgent assessment.

Do not replace missed doses or restart after a toxicity hold without instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

5 Blood / infection

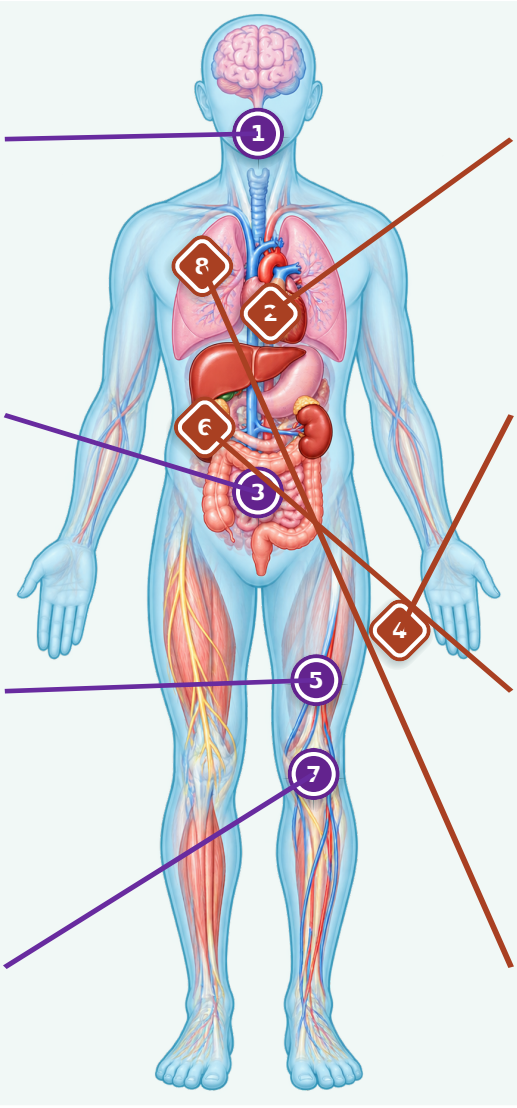
Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

7 Energy / daily tasks

Treatment effects

Report fatigue that prevents usual activities or self-care.



2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

6 Hydration / urine

Capecitabine safety

Reduced urine, dizziness or inability to drink needs assessment.

8 Unexpected early illness

Capecitabine

Severe early diarrhoea, mouth soreness or sudden illness needs urgent review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Keep your treatment details to hand

Colorectal and breast cancer · NCCP 00216 v8 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Capecitabine

Twice daily Days 1–14; no capecitabine Days 15–21. Confirm total course with team.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.